

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: JonesPeriod: April 1 to April 30, 2026Report Date: May 1, 2026Coverage: 510/720h (71%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

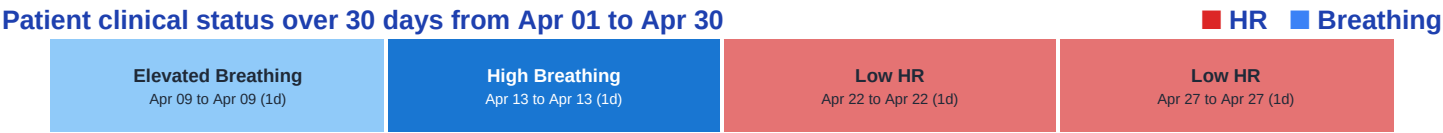
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	55 bpm	44 bpm	73 bpm
Breathing	17 brpm	11 brpm	29 brpm

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

5 episodic events Detected, spanning 5h total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
5	5h	Elevated Breathing	<1	71%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 13	11 PM to 12 AM	1h	1	High Breathing	31	8	48	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Apr 22	4 AM to 5 AM	1h	1	Low Heart Rate	44	42	47	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 27	11 PM to 12 AM	1h	1	Low Heart Rate	44	42	50	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 09	2 PM to 3 PM	2h	2	Elevated Breathing	26	13	35	Check SpO2 and lung sounds; assess for fluid overload or early infection

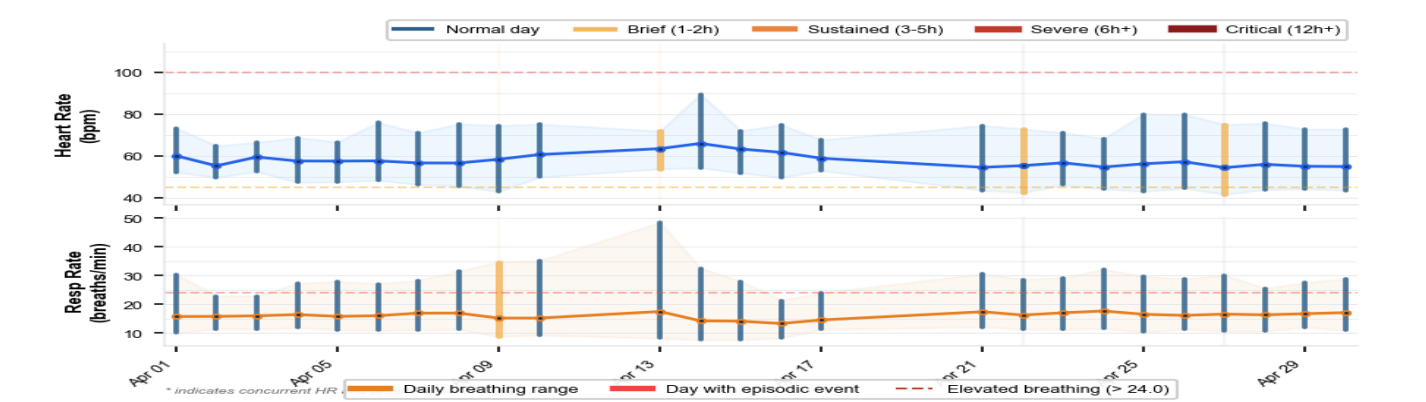
30% of readings are low confidence.

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 35 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 31 brpm, peak 48 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Low Heart Rate (avg 44 bpm, min 42 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 4 unstable phases with 14 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

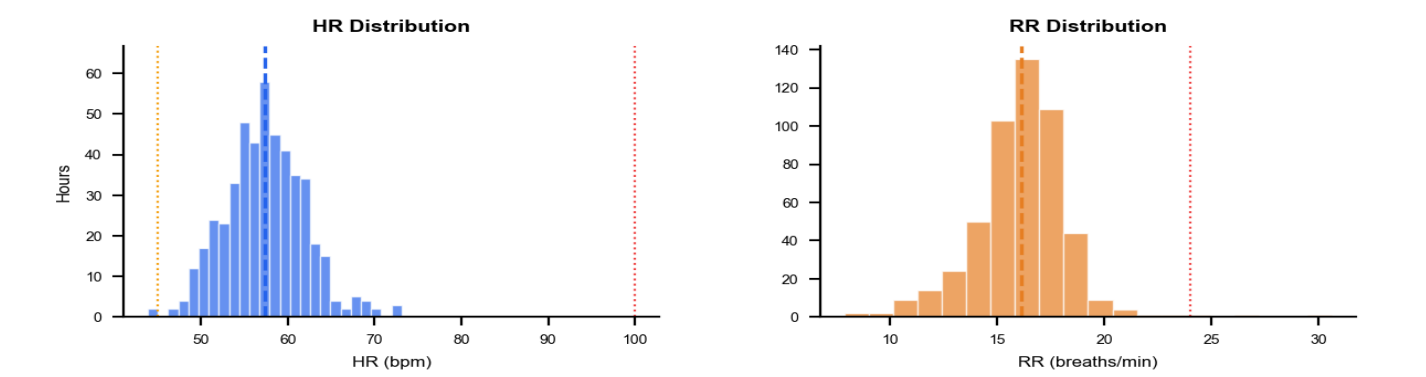
Clinical Pattern Observations:

- 1. Clustered pattern: Episodic events on 4 of 30 days (13%).
- 2. Nocturnal pattern: 3 of 5 eligible episodes during evening or night hours.

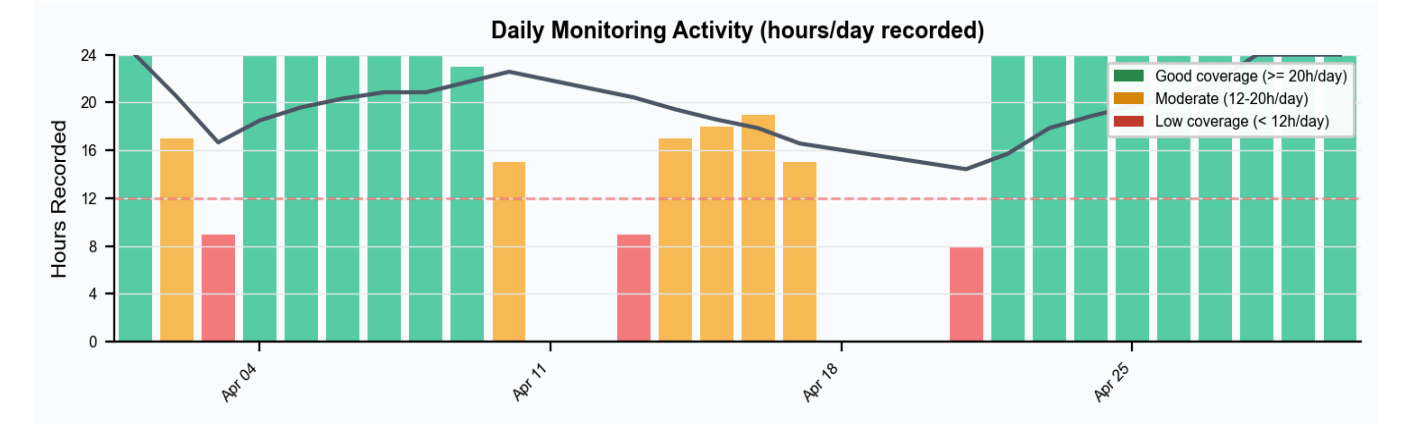


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.